

Shock & Haemorrhage

1. Regarding hypovolaemic shock, it occurs in which of the following conditions?

A. Pump failure

B. Reduction in intravascular volume

C. Disturbances in oxygen delivery and oxygen consumption

D. Exposure to an allergen

2. Which of the following conditions cause septic shock?

A. Radiological contrast media

B. Injury to the thoracic spinal cord

C. Arrhythmias

D. **Burns**

3. Which of the following diseases causes septic shock?

A. **Pancreatitis**

B. Heart valve dysfunction

C. Cardiac tamponade

D. Pulmonary embolism

4. Capillary endothelial injury occurs in which type of shock?

A. Cardiogenic shock

B. Neurogenic shock

C. Septic shock

D. Anaphylactic shock

5. Cardiac output typically increases in which type of shock?

A. Haemorrhagic shock

B. Septic shock

C. Anaphylactic shock

D. Cardiogenic shock

6. Mitochondrial dysfunction occurs due to which of the following conditions?

A. Anaphylactic shock

B. Septic shock

C. Neurogenic shock

D. Cardiogenic shock

7. Which of the following is the commonest site of infection related sepsis?

A. Skin

B. Lungs

C. Abdomen

D. Urinary tract

8. In which of the following conditions formation of microthrombi within the microcirculation commonly occurs?

A. Anaphylactic shock

B. Neurogenic shock

C. Septic shock

D. Cardiogenic shock

9. Cytokine-mediated proinflammatory response is pathognomonic for which condition?

A. Hypovolaemic shock

B. Septic shock

C. Cardiogenic shock

D. Anaphylactic shock

10. Which parameter is present in SIRS?

A. WBC count ; 6000

B. BP; 130/ 70 mmHg

C. RR,; 30/ min

D. HR; 80/ min

11. Mean arterial blood pressure (MAP) is equal to ;

- A. The product of cardiac output (CO) and systemic vascular resistance (SVR).
- B. The sum of cardiac output and SVR
- C. The product of SVR and stroke volume
- D. Systolic BP divided by diastolic BP.

12. Oxygen consumption equals;

- A. $CO \times HGB \times 1.34 \times (SaO_2 - SvO_2)$
- B. $CO \times HGB \times 1.34 \times SaO_2 / 100$
- C. $CO \times HGB \times 1.34 \times SvO_2$
- D. $CO \times HGB \times 1.34 \times SaO_2$

13. Catecholamines are released in which of the following conditions?

- A. Hypovolaemic shock
- B. Anaphylactic shock
- C. Cardiogenic shock
- D. Septic shock

14. Renin Angiotensin-II is released in ;

- A. Cardiogenic shock
- B. Hypovolaemic shock
- C. Anaphylactic shock
- D. Septic shock

15. Catecholamines are released from;

- A. Adrenal medulla
- B. Kidney
- C. Renal tubules
- D. Hypothalamus

16. $10 \text{ Glucose} + 24 \text{ O}_2 + \text{ADP} + \text{P} \rightarrow \text{H}_2\text{O} + 24 \text{ CO}_2 + \text{HEAT} + 360 \text{ ATP}$

What is the type of process?

A. Oxidative phosphorylation

B. Non oxidative phosphorylation

C. Glycogenesis

D. Lipogenesis

17-Which of the following products does not enter to Krebs cycle?

A. Glucose

B. Lactate

C. Pyruvate

D. Acetyl CoA

19. When oxygen delivery falls, cells are able to increase the proportion of oxygen extracted from the blood, but this compensatory mechanism is limited, with a maximal oxygen extraction ratio of about ;

A. 25%

B. 40%

C. 50 %

D. 75%

20. The patient with shock can be manifested by which of the following?

A. Decreased systolic BP that results in decreased coronary flow

B. Decreased ventilation/ perfusion mismatch

C. Metabolic alkalosis

D. Metabolic acidosis

21. Which of the following is clinical effect of shock?

A. Renal hypoperfusion results in inactivated renin angiotensin system

B. Increased shunt

C. Intact mucosal barrier

D. Acute pulmonary edema

22. It is common practice to use blood to maintain a haemoglobin concentration. What is the level of accepted haemoglobin?

A. 7 g/dl

B. 8 g/dl

C. 9 g/dl

D. 10 g/dl

23. During management of shock, fluid response is manifested by ;

A . increased tachycardia

B. Decreased blood pressure

C. Improving peripheral perfusion

D. Decreased urine output

24. During management of shock, once parameters stop improving it is unlikely that further fluid will be beneficial, particularly if there is an associated fall in oxygen saturation and the development of pulmonary oedema. When do we stop fluids?

A. Increased preload and increased stroke volume

B. Decreased preload and increased stroke volume

C. Decreased preload and decreased stroke volume

D. Increased preload and decreased stroke volume

25. What is the drug that acts on both myocardial contractility and systemic vascular resistance?

A. Noradrenaline

B. Adrenaline

C. Dopamine

D. Dobutamine

26. Noradrenaline is characterized by;

A. Inotrope

B. Increased myocardial contractility

C. Increased stroke volume

D. Increases systemic vascular resistance

27. Dobutamine acts to overcome shock through;

A. Increases systemic vascular resistance

B. Vasopressor effect

C. Positive inotropic effect

D. Decreased heart rate

28. What is the normal adult blood volume in relation to body weight,

A. 5 %

B. 6 %

C. 7 %

D. 8 %

29. In a 100 kg man, what is the estimated blood volume (EBV)?

A. 5000 ml

B. 6000 ml

C. 7000 ml

D. 8000 ml

30.;The severity of haemorrhagic shock is frequently classified according to percentage of EBV lost. What is a compensated state of blood loss?

A. 10%

B. 20%

C. 25%

D. 30%

31. What is the immediately life-threatening Haemorrhage as a proportion of body weight?

A. 15%

B. 20%

C. 30%

D. 40%

32. Which of the following describes the term ‘massive haemorrhage?’

- A. Loss of EBV in 24 hours;**
- B. Loss of 50% EBV in 12 hours
- C. Blood loss at a rate ≥ 50 ml/min
- D. Loss of 30% of EBV in 9 hrs

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HERNIA

1.What is the most common type of hernia?

- A. Indirect inguinal hernia**
- B. Direct inguinal hernia
- C. Femoral hernia
- D. Epigastric hernia

2.What is the most common hernia in female?

- . A Indirect inguinal hernia**
- B. Direct inguinal hernia
- C. Femoral hernia
- D. Epigastric hernia

3. Failure of the processus vaginalis to obliterate after birth leads to which type of hernia?

- A. Direct inguinal hernia
- B. Indirect inguinal hernia**
- C. Umbilical hernia
- D. Femoral hernia

4. Which one of the following hernia is more liable to obstruction?

- A. Direct inguinal hernia
- B. Indirect inguinal hernia
- C. Umbilical hernia

D. Femoral hernia

5. Which of the following hernias is less liable to strangulation?

- A. Direct inguinal hernia
- B. Indirect inguinal hernia

C. Umbilical hernia

D. Femoral hernia

6. Which of the following hernias is less liable to strangulation?

A. Direct inguinal hernia

- B. Pantaloon hernia
- C. Indirect inguinal hernia
- D. Femoral hernia

7. Which of the following hernias is more liable to irreducibility?

- A. Direct inguinal hernia
- B. Paraumbilical hernia
- B. Indirect inguinal hernia

D. Femoral hernia

8. What is the main complication occurs in paraumbilical hernia?

- A. Obstruction
- B. **Irreducibility**
- C. Strangulation
- D. Infection

9. What is the cause of sepsis in patient with hernia?

- A. Obstruction
- B. Irreducibility
- C. Strangulation
- D. Recurrence

10. What is the most dangerous complication of hernia?

- A. Obstruction
- B. Irreducibility
- C. Strangulation
- D. Recurrence

11. Where is the internal (deep) inguinal ring is located?

- A. 1 cm above the mid-inguinal point
- B. 1 cm above the midpoint of inguinal ligament
- C. At the pubic tubercle
- D. At the anterior superior iliac spine

12. Where is the superficial ring located?

- A. Below and medial to the pubic tubercle.
- B. Above and medial to the pubic tubercle
- C. Above and lateral to the pubic tubercle
- D. Below and lateral to the pubic tubercle

13. Which if the following arteries is not from the contents of the spermatic cord?

- A. Artery of the vas
- B. Testicular artery
- C. Femoral artery
- D. Cremasteric artery

14. Which of the following veins is from contents of the spermatic cord?

- A. Femoral vein
- B. Testicular vein
- C. Pampiniform plexus of veins
- D. Cremasteric vein

15. In children, repair of inguinal hernia is done through?

- A. Herniotomy
- B. Herniorrhaphy
- C. Hernioplasty
- D. Synthetic mesh

16. In 30 year old male, inguinal hernia repair is done through which of the following?

- A. Herniotomy only
- B. Herniotomy & Herniorrhaphy
- C. Herniotomy, Herniorrhaphy & Hernioplasty
- D. Herniotomy & suture repair

17. What is the lateral structure to femoral ring?

- A. Inguinal ligament
- B. Femoral vein
- C. Pectineal ligament
- D. Lacunar ligament

18. What is the main content in the femoral canal?

- A. Femoral artery
- B. Femoral vein
- C. Lymph nodes
- D. Femoral nerve

19. What is the anatomical relation of femoral sheath to femoral ring?

A. Lies lateral to femoral ring

B. Lies medial to femoral ring

C. Lies above the femoral ring

D. Lies below the femoral ring

20. Which of the following structures is pulsatile swelling ?

A. Saphena varix

B. Femoral artery aneurysm

C. Femoral hernia

D. Psoas abscess

21. Which of the following structures disappears on pressure?

A. Saphena varix

B. Femoral artery aneurysm

C. Femoral hernia

D. Psoas abscess

22. Which of the following structures disappears on lying down ?

A. Saphena varix

B. Femoral artery aneurysm

C. Femoral hernia

D. Psoas abscess

23. Which of the following structures can be reduced easily?

A. Saphena varix

B. Femoral artery aneurysm

C. Obstructed femoral hernia

D. Psoas abscess

24. Which of the following hernias is considered as inguinoscrotal swelling?

A. Direct inguinal hernia

B. Indirect inguinal hernia

C. Femoral hernia

D. Incisional hernia

25. In a 40-year obese multiparous women, what is the most probable type of hernia can develop?

A. Epigastric hernia

B. Femoral hernia

C. Paraumbilical hernia

D. Incisional hernia .

26. What is the most likely type of hernia to occur in 30-year old male patient after 2 years of appendectomy?

A. Femoral hernia

C. Paraumbilical hernia

C. Incisional hernia

D. Epigastric hernia

27. A 65- year old male presents with features of pain, vomiting, and abdominal distension. He is known to have left oblique inguinal hernia. What is the best urgent intervention for him?

A. Reduce the hernia

B. Give analgesia

C. Laparoscopic exploration

C. Do abdominal sonar

Wounds

1. Which of the following carries higher risk of disease transmission?
 - A. Contusion
 - B. Abrasion
 - C. Puncture wound
 - D. Laceration
2. Which of the following wounds has tidy edges?
 - A. Contusion
 - B. Abrasion
 - C. Incision
 - D. Laceration
3. Collection of blood in a potential space describes which of the following wounds?
 - A. Compartment syndrome
 - B. Contusion
 - C. Abrasion
 - D. Hematoma
4. Which of the following lead to acute renal failure?
 - A. Compartment syndrome
 - B. Contusion
 - C. Abrasion
 - D. Crush injury
5. In contaminated wounds, which of the following should be done?
 - A. Do nerve repair
 - B. Do tendon repair
 - C. Close the fascia
 - D. Ligate the blood vessel
6. Secondary sutures are done in which of the following conditions?
 - A. Infected wound waiting to granulate till it gets clean.
 - B. At the time of injury with recent clean wound.
 - C. Doubtful wounds
 - D. Late presentation after injury.
7. Which of the following properties describes the ideal wound dressing ?
 - A. Allergic.
 - B. Non-traumatic on removal.
 - C. Permeable to bacteria.
 - D. Do not allow gaseous exchange.

8. Semi-occlusive dressing is described as :

- A. **Permeable to water.**
- B. Not suitable for infected wounds.
- C. Permeable to bacteria.
- D. Do not allow gaseous exchange.

9. What is the main cell in wound healing?

- A. Neutrophils
- B. Mast cells
- C. **Macrophages**
- D. Basophils

10. Which vitamin is essential for normal collagen formation?

- A. Vitamin A
- B. Vitamin B
- C. **Vitamin C**
- D. Vitamin D

Short notes:

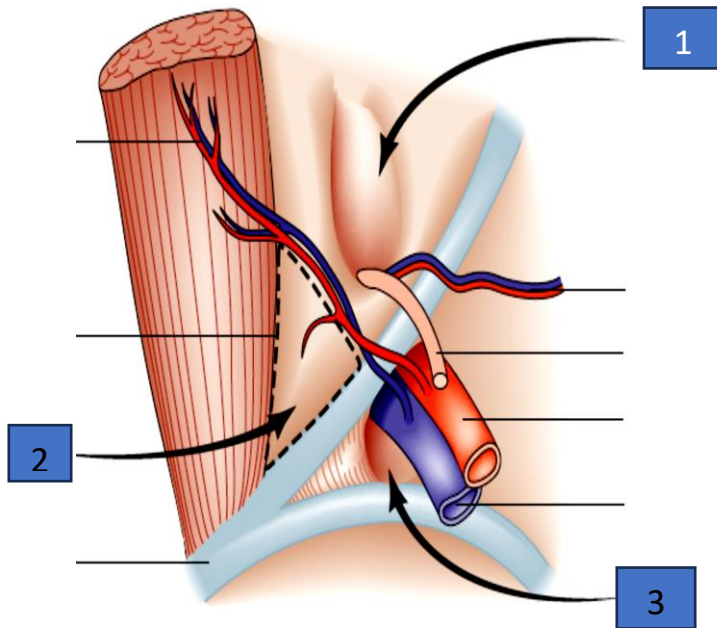
What are the early and late complications of wound healing”

What are the stages of wound healing?

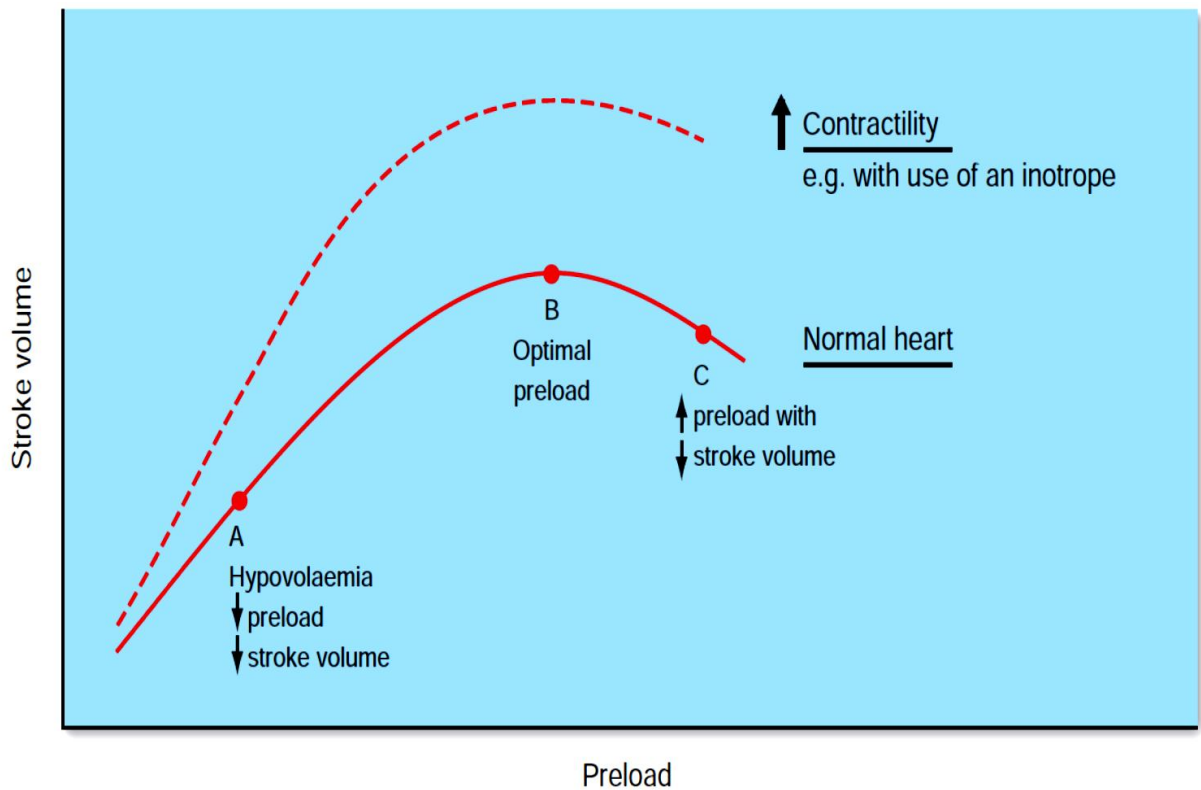
Mention the responses with inflammatory phase in wound healing

Mention the 4 mechanisms with proliferative phase in wound healing

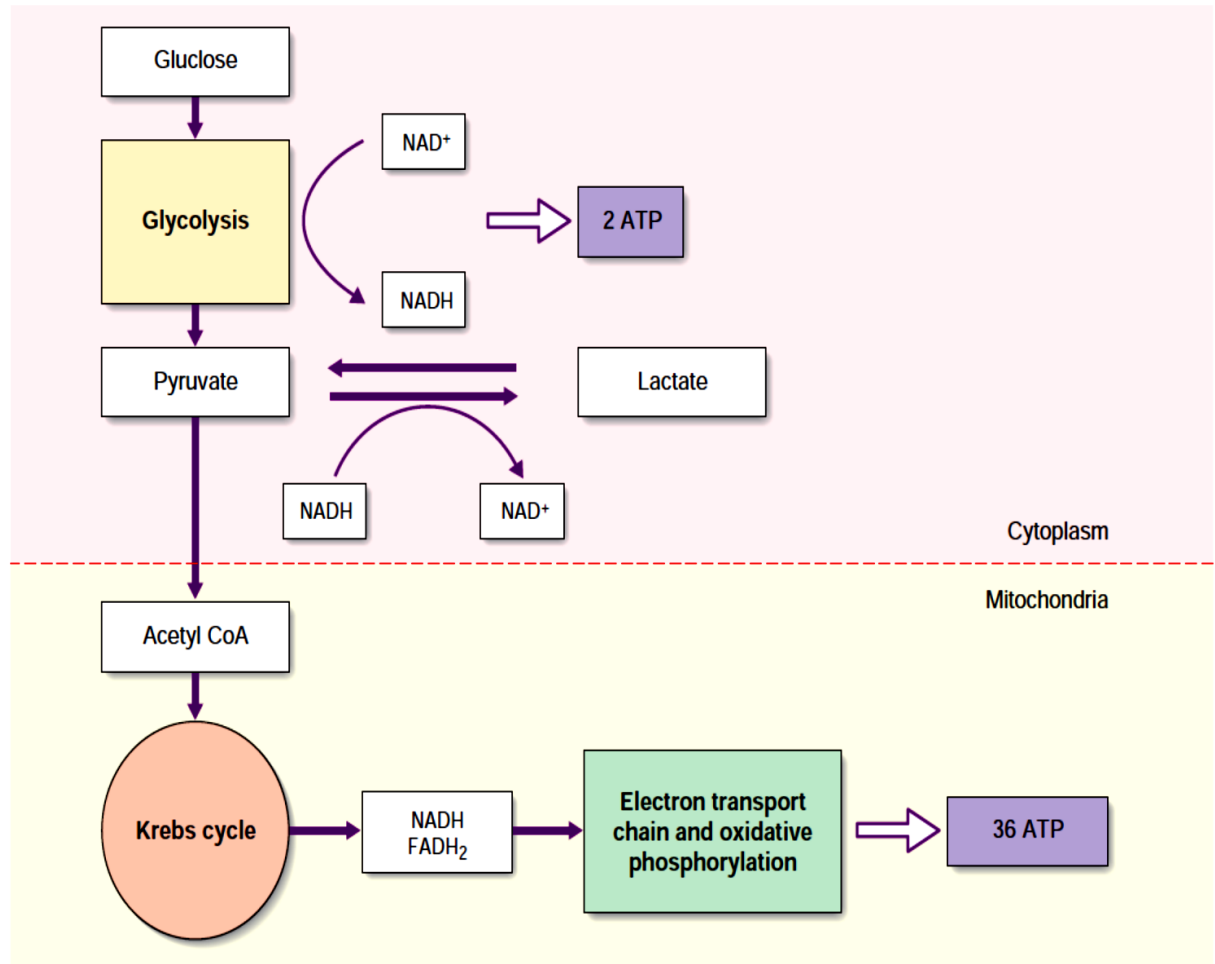
What are the types of chronic wounds?

Short Answers**Regarding the above diagram;**

1. Mention the name of each number.
2. What is number for the most common type of hernia? 1
3. Which type is more liable to obstruction? 3
4. Which type occurs more in females? 1
5. Which type descends to the scrotum? 1



- 1-At which point A, B, or C vasopressor drugs must be initiated? C
- 2-Mention one drug which increases both SVR and cardiac output. Adrenaline



1-What is the net-result of aerobic pathway, regarding ATP production?

Each mole of glucose gives 38 moles of ATP

2-In hypoxia, what is the substrate produced and causes muscle fatigue?

Lactate



- 1-What is the name of this lesion? **Paraumbilical hernia**
 - 2-What is the predisposing factor? **Multiparous woman**
 - 3-What is the complication? **Irreducibility**
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- 1-What is the name of this intervention? **Fasciotomy**
- 2-What is its benefit? **To relieve compartmental syndrome**



What is the name of this lesion? Haematoma

How can you treat it? Evacuation

Mention its complications. Infection and calcification

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What is the name of this wound?

.....Degloving injury.....



1. What is the name of this wound? Surgical Incision
2. What is the type of wound healing here? 1ry intention

Acute Appendicitis& Intestinal obstruction

1. What is the surface anatomy of appendix?

- A. At ASIS
- B. At the umbilicus
- C. Point at outer 1/3 between ASIS & Umbilicus
- D. Point at inner 2/3 between ASIS & Umbilicus

2. What is the most common cause of acute abdomen?

- A. Intestinal obstruction
- B. Acute appendicitis
- C. Acute cholecystitis
- D. Gastroenteritis

3. What is the most common position of appendix?

- A. Paracecal
- B. Retrocecal
- C. Pelvic
- D. Paraileal

4. Where does appendicular pain begin first?

- A. Central
- B. Rt iliac fossa
- C. Left iliac fossa
- D. Epigastric

5. Where is the maximal point of tenderness in acute appendicitis?

- A. Around the umbilicus
- B. At ASIS
- C. Mc Burney's
- D. Lt iliac fossa

6. Which of the following is appropriate for diagnosis of acute appendicitis?

- A. Clinical examination is enough
- B. Abdominal ultrasound is required before surgery
- C. Abdominal CT is mandatory for diagnosis
- D. MRI is a common diagnostic tool

7. Rovsing sign is described as?

- A. Tenderness on palpation of Rt iliac fossa
- B. Tenderness on palpation of left iliac fossa
- C. Rt iliac fossa tenderness on palpation of Lt iliac fossa
- D. Lt iliac fossa tenderness on palpation of Rt iliac fossa

8. Psoas stretch sign is positive when:

- A. Painful extension of Lt hip
- B. Painful extension of Rt hip
- C. Painful flexion of Lt hip
- D. Painful flexion of Rt. hip

9. Which of the following indicates peritonitis?

- A. Tender Mc Burney's point
- B. Rebound tenderness
- C. Rovesi~~ng~~ sign
- D. Abdominal guarding

10. What is the DD of pain in Rt iliac fossa

- A. Acute appendicitis, diverticulitis, acute cholecystitis
- B. Acute appendicitis, appendicular abscess, Lt ovarian cyst
- C. Acute appen~~dic~~icitis, Rt ureteric stone, rupture of Rt ovarian cyst
- D. Acute appendicitis, acute cholecystitis, Pyloric stenosis

11. Conservative management is applied in which of the following conditions?

- A. Acute appendicitis
- B. Appen~~d~~icular mass
- C. Peritonitis
- D. Strangulated hernia

12. An interval appendicectomy is indicated in which of the following?

- A. Acute appendicitis
- B. Appen~~d~~icular mass
- C. Peritonitis
- D. Gangrenous appendicitis

13. Which of the following is the aetiology of paralytic ileus?

- A. Interference of blood supply of the gut
- B. Mechanical obstruction
- C. Functional ob~~stru~~ction
- D. Strangulation

14. Strangulation as a cause of intestinal obstruction occurs with?

- A. Mechanical obstruction
- B. Functional obstruction
- C. Intestinal ischemia
- D. Faecal impaction

15. Which of the following is less liable to cause strangulation?

- A. Volvulus
- B. Intussusception
- C. Cancer rectum
- D. MVO

16. Which of the following is not a cause of intramural acute intestinal obstruction?

- A. Tumors
- B. Chronic diverticulitis
- C. Gall stone ileus
- D. Crohn's disease.

17. A 50-year old man is presented to emergency department with abdominal colic and vomiting.

He didn't pass stools 2 days ago & there is no fever. What do you expect this patient have?

- A. Acute cholecystitis
- B. Acute appendicitis
- C. Strangulated hernia
- D. Paralytic ileus

18. In which of the following causes of intestinal obstruction there is absent intestinal sounds?

- A. Strangulated hernia
- B. Intussusception
- C. Paralytic ileus
- D. Volvulus

19. A 40-year old man is presented to emergency department with abdominal pain, he is diagnosed as intestinal obstruction, Neither NG tube nor enema could relieve his pain, there is abdominal tenderness and rigidity.

What is the diagnosis of this condition?

- A. Sigmoid volvulus
- B. Intussusception
- C. Adhesive intestinal obstruction

D. Strangulated loop

20. During initial evaluation of a patient who is diagnosed as intestinal obstruction, if there is no signs of peritonitis, what will you proceed?

- A. Emergency surgery is mandatory
- B. Conservative management for one week and if not improved go for surgery
- C. Conservative management for one day and discharge home
- D. **Conservative management for 24 hrs** and if not improved go for surgery

21. What is the best management of a patient who underwent difficult surgery before and now is presented by acute intestinal obstruction?

A. Conservative treatment

- B. Elective surgery
- C. Emergency laparotomy
- D. Emergency laparoscopy

22. Which of the following patients need emergency surgery?

- A. Patient with adhesive intestinal obstruction
- B. Patient with cancer colon

C. Patient with peritonitis

- D. Patient with history of previous difficult surgery

23. What does the term incarcerated hernia mean?

- A. Intestinal segment is entrapped with ischemia
- B. Intestinal segment is entrapped without ischemia**
- C. Intestinal segment is reducible but ischemic
- D. Intestinal segment is ischemic and irreducible

24. What is the best treatment for incarcerated hernia?

- A. Urgent surgery
- B. Elective surgery
- C. Conservative management
- D. Examine for viable intestine**

25. What is the best management of Lt-sided large bowel obstruction?

- A. Resection and direct anastomosis
- B. Resection with colostomy**
- C. Resection without colostomy
- D. Resection with ileostomy

Haemorrhoids & Fistula

Short notes

What are the risk factors for piles?

Mention the vessels causing piles.

What are the types of Haemorrhoids?

What are the 3 clinical features of Haemorrhoids?

What is the classification of piles according to prolapse?

True or False

3rd degree piles can be reduced spontaneously ×

4th degree piles can't be reduced manually ✓

MCQs

- 1. Flexible sigmoidoscopy or colonoscopy may be appropriate if there is concern about the cause of symptoms; haemorrhoids rarely start over the age of 55y and it is often best to assume another cause until proven otherwise in these cases.**

What is cause of piles in old patients above 55 years ?

A. Cancer rectum

B. Crohn's disease

C. Multiparous lady

D. Anemia

- 2. Flexible sigmoidoscopy is preferred in cases with bleeding per rectum in which of the following patients?**

A. Young age 25 years old with Hemorrhoids

B. 40 year old with repeated pregnancies

C. 69 years old man

D. Obese patient

- 3. What is the best management for 4th grade piles?**

A. Laxatives

B. Phenol injection

C. Band ligation

D. Haemorrhoidectomy

- 5. What is the most common type of fistula in ano?**

A. Transsphincteric

B. Intersphincteric

C. Suprasphincteric

D. Extrasphincteric

6. What is the most sensitive method of determining the course of the fistulous track in the perianal fistula?

- A. Probing
- B. MRI scanning**
- C. Ultrasound
- D. Flexible sigmoidoscopy

7. Seton sutures are applied for which of the following conditions?

- A. High fistula**
- B. Low fistula
- C. Complicated fistula
- D. Piles

MISCELLANEOUS MCQs

1. What is the most common type of hernia found in a 30-year old man?

- A. Indirect inguinal hernia
- B. Direct inguinal hernia**
- C. Femoral hernia
- D. Epigastric hernia

2. Which one of the following hernia is more liable to strangulation?

- A. Direct inguinal hernia
- B. Indirect inguinal hernia**
- C. Umbilical hernia
- D. Incisional hernia

3. In a 40-year obese multiparous women, what is the most probable type of hernia can develop?

- A. Epigastric hernia
- B. Spigelian hernia
- C. Paraumbilical hernia**
- D. Incisional hernia .

4. Which of the following diseases causes septic shock?

- A. **Pancreatitis**
- B. Heart valve dysfunction
- C. Cardiac tamponade
- D. Pulmonary embolism

5. In which type of shock myocardial contractility increases as a compensatory mechanism ?

- A. **Haemorrhagic shock**
- B. Septic shock
- C. Anaphylactic shock
- D. Cardiogenic shock

6. The patient with shock can be manifested by which of the following?

- A. Decreased systolic BP that results in decreased coronary flow
- B. **Increased ventilation/ perfusion mismatch**
- C. Metabolic alkalosis
- D. Respiratory acidosis

7. What is the main cell in wound healing?

- A. Neutrophils
- B. **Macrophages**
- C. Mast cells
- D. Basophils

8. The ideal wound dressing should have which of the following properties?

- A. **Maintain a moist environment.**
- B. Keeps excess exudates.
- C. Doesn't allow gaseous exchange.
- D. Permeable to bacteria.

1. What is the most common hernia in female?

A. Indirect inguinal hernia

B. Direct inguinal hernia

C. Femoral hernia

D. Epigastric hernia

2. Which one of the following hernia is more liable to obstruction?

A. Direct inguinal hernia

B. Indirect inguinal hernia

C. Umbilical hernia

D. Femoral hernia

3. In children, repair of inguinal hernia is done through which of the following methods?

A. Herniotomy

B. Herniorrhaphy

C. Hernioplasty

D. Synthetic mesh

4. In which type of shock myocardial contractility increases as a compensatory mechanism ?

A. Haemorrhagic shock

B. Septic shock

C. Anaphylactic shock

D. Cardiogenic shock

5. The patient with shock can be manifested by which of the following?

A. Decreased systolic BP that results in decreased coronary flow

B. Decreased ventilation/ perfusion mismatch

C. Metabolic alkalosis

D. Metabolic acidosis

6. Which type of wounds causes transmission of hepatitis-C virus?

A. Contusion

B. Laceration

C. Puncture wound

D. Gunshots

7. When there is collection of blood in a potential space, what is the type of wound?

A. Laceration

B. Haematoma

C. Degloving injury

D. Crush injury

8. The ideal wound dressing should have which of the following properties?

A. Maintain a dry environment.

B. Keeps excess exudates.

C. Doesn't allow gaseous exchange.

D. Impermeable to bacteria.

Short notes

What is the classification of perianal fistula?

What is the most common type of fistula in ano?

What are the clinical features of perianal fistula?

Breast cancer

1. What is the percentage of early screening to detect breast cancer?

- A. 40%
- B. 50%
- C. 60%
- D. 70%

2. Ductal adenocarcinoma of breast represents which percentage of all breast pathologies?

- A. 50 %
- B. 60 %
- C. 70 %
- D. 80 %

3. What is the percentage of oestrogen progesterone receptors expression in breast cancer?

- A. 50 %
- B. 60 %
- C. 70 %
- D. 80 %

4. What is the commonest breast cancer presentation?

- A. Bleeding per ducts
- B. Breast lump
- C. Breast pain
- D. Metastatic

5. What is the meaning of tethering in breast cancer?

- A. Lymphoedema of the skin
- B. Lump is attached to surrounding breast tissue or skin
- C. Metastatic mass
- D. Axillary lymph nodes infiltrated

6. Nipples may be affected by an underlying cancer in the form of what?

- A. Eversion
- B. Deviation laterally
- C. **Inversion**
- D. Pigmentation

7. In a young female with breast cancer, what is the best radiologic test to establish diagnosis?

- A. Mammogram
- B. **Sonography**
- C. FNAC
- D. Core biopsy

8. When the surgeon targets breast-conserving surgery in adequate size breast and tumour location not central. What is the best surgery?

- A. **Wide local excision**
- B. Simple mastectomy
- C. Axillary lymph node clearance
- D. Sentinel lymph node dissection

9. What is the best local treatment with cosmetic result for large breast tumours?

- A. Wide local excision
- B. **Simple mastectomy**
- C. Axillary lymph node clearance
- D. Sentinel lymph node dissection

10. What is the best management for centrally located with late presentation ulcerated breast cancer?

- A. Wide local excision
- B. **Simple mastectomy**
- C. Axillary lymph node clearance
- D. Sentinel lymph node dissection

11. What is the best surgery used for multifocal tumours with evidence of widespread in situ changes?

- A. Wide local excision
- B. Simple mastectomy**
- C. Axillary lymph node clearance
- D. Sentinel lymph node dissection

Management of Cancer Patients

1. Regarding transitional cell carcinoma, what is the female to male incidence?

- A. 1:3**
- B. 2:3
- C. 3:1
- D. 3:2

2. Which of the following pathological types of transitional cell carcinoma is associated with schistosomiasis?

- A. Carcinoma in situ
- B. Adenocarcinoma
- C. Squamous cell carcinoma**
- D. Superficial carcinoma

3. What is the most common presentation in transitional cell carcinoma?

- A. Painful haematuria
- B. Painless haematuria**
- C. Dysuria
- D. Frequency

4. Which of the following is invasive diagnostic tool in transitional cell carcinoma?

- A. Ultrasound
- B. CT scan
- C. Cystoscopy
- D. MRI

5. Which of the following permits examination of the renal cortex and can detect tumours of 1 cm diameter in the pelvicalyceal system, ureter, and bladder?

- A. Ultrasound
- B. Cystoscopy
- C. Urine analysis
- D. Plain abdomen X-ray

6. Regarding TCC, immunotherapy with intravesical BCG is effective in which of the following conditions?

- A. Superficial type
- B. Carcinoma in situ
- C. Adenocarcinoma
- D. Squamous cell carcinoma

7. Which of the following is radio-and chemo-resistant?

- A. Superficial type
- B. Carcinoma in situ
- C. Adenocarcinoma
- D. Invasive type

8. What is nephrostomy ?

- A. A small tube inserted into the bladder through the skin of the lower abdomen
- B. A tiny incision and inserts a small tube, through the skin of the back into the kidney.
- C. Inserting a thin, flexible tube into the bladder to drain urine.
- D. A thin flexible tube that is inserted into the ureter between the kidney and the bladder.

9. What the ureteral stent?

- A. A small tube inserted into the bladder through the skin of the lower abdomen
- B. A tiny incision and inserts a small tube, through the skin of the back into the kidney.
- C. Inserting a thin, flexible tube into the bladder to drain urine.
- D. A thin flexible tube that is inserted into the ureter between the kidney and the bladder.

10. What does the term urostomy refer to?

- A. A stoma in the abdomen that connects to the urinary tract to allow urine to drain freely.
- B. A small tube inserted into the bladder through the skin of the lower abdomen
- C. A tiny incision and inserts a small tube, through the skin of the back into the kidney.
- D. Inserting a thin, flexible tube into the bladder to drain urine.

11. Rapid bowel preparation is not indicated in which of the following conditions?

- A. Intestinal Obstruction
- B. Intestinal Perforation
- C. Hernia repair
- D. Massive bleeding from the colon

12. When doing anastomosis between bowel ends, it shouldn't be?

- A. Adequately vascularized
- B. Sutured with tension.
- C. Water-tight
- D. Gas tight

13. Which of the following is from the characteristics of the colostomy?

- A. There are 2 loops
- B. Present on left abdominal wall
- C. Present on right abdominal wall
- D. Done with cancer caecum resection

14. Mechanical preparation for the colon is indicated in what condition?

- A. Intestinal obstruction
- B. Massive bleeding from the colon
- C. Intestinal perforation
- D. Cancer caecum

15. Chemical preparation of the colon includes?

- A. Neomycin
- B. Laxatives
- C. Enema
- D. Low residue diet

16. Mechanical preparation of the colon includes?

- A. Bowel irrigation with crystalloid
- B. Cephalosporins
- C. Metronidazole
- D. Neomycin

17. In which of the following methods of urinary diversion the urine flows via the urethra?

- A. Supra pubic cystostomy
- B. Bladder substitute
- C. Ileal conduit
- D. Cutaneous ureterostomy

18. When there is a stone in the renal pelvis, what is the preferred method for urine diversion?

- A. Nephrostomy
- B. Suprapubic cystostomy
- C. Folley catheter
- D. Ileal conduit

19. What is the best method to divert urine in ureteral stone?

- A. Foley catheter
- B. Double J stent**
- C. Nephrostomy tube
- D. Suprapubic cystostomy

Breast cancer

1. What is the percentage of early screening to detect breast cancer?

- A. 40%**
- B. 50%
- C. 60%
- D. 70%

2. Ductal adenocarcinoma of breast represents which percentage of all breast pathologies?

- A. 50 %
- B. 60 %
- C. 70 %
- D. 80 %**

3. What is the percentage of oestrogen progesterone receptors expression in breast cancer?

- A. 50 %
- B. 60 %
- C. 70 %**
- D. 80 %

4. What is the commonest breast cancer presentation?

- A. Bleeding per ducts
- B. Breast lump**
- C. Breast pain
- D. Metastatic

5. What is the meaning of tethering in breast cancer?

- A. Lymphoedema of the skin
- B. Lump is attached to surrounding breast tissue or skin
- C. Metastatic mass
- D. Axillary lymph nodes infiltrated

6. Nipples may be affected by an underlying cancer in the form of what?

- A. Eversion
- B. Deviation laterally
- C. Inversion
- D. Pigmentation

7. In a young female with breast cancer, what is the best radiologic test to establish diagnosis?

- A. Mammogram
- B. Sonography
- C. FNAC
- D. Core biopsy

8. When the surgeon targets breast-conserving surgery in adequate size breast and tumour location not central. What is the best surgery?

- A. Wide local excision
- B. Simple mastectomy
- C. Axillary lymph node clearance
- D. Sentinel lymph node dissection

9. What is the best local treatment with cosmetic result for large breast tumours?

- A. Wide local excision
- B. Simple mastectomy
- C. Axillary lymph node clearance
- D. Sentinel lymph node dissection

10. What is the best management for centrally located with late presentation ulcerated breast cancer?

A. Wide local excision

B. Simple mastectomy

C. Axillary lymph node clearance

D. Sentinel lymph node dissection

11. What is the best surgery used for multifocal tumours with evidence of widespread in situ changes?

A. Wide local excision

B. Simple mastectomy

C. Axillary lymph node clearance

D. Sentinel lymph node dissection

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Operative Questions

Short notes

1. Mention 3 complications of laparoscopic cholecystectomy.
2. Enumerate 3 indications of laparoscopic cholecystectomy
3. How to prepare the patient who undergo laparoscopic cholecystectomy? Mention 3 points
4. What are the uses of Verres needle ?
5. Mention 3 indications for thyroidectomy.
6. Mention 3 complications of thyroidectomy.
7. What are the nerve complications related to thyroidectomy?
8. How do you prepare the patient for thyroidectomy?
9. What are the nerves must be cautious for during thyroidectomy?
10. What is the electrolyte affected during thyroidectomy?

MCQs

1. Which of the following structures is NOT from the boundaries of Calot triangle?

- A. Cystic duct
- B. Common hepatic duct

C. Common bile duct

- D. Undersurface of the liver

2. What does Charcot's triade refer to?

- A. Pain, jaundice and dyspnea

B. Pain, jaundice and fever

- C. Pain, jaundice and vomiting
- D. Pain, jaundice and diarrhoea

3. What is the gas used for insufflation during laparoscopic cholecystectomy?

- A. Oxygen

B. Carbon dioxide

- C. Hydrogen
- D. Nitrogen

4. Which of the following nerves is NOT related to thyroid gland?

- A. External laryngeal n
- B. Internal laryngeal n

C. Glossopharyngeal n

- D. Recurrent laryngeal n.

5. Which of the following nerves is found at the lower pole of thyroid gland?

- A. External laryngeal n
- B. Internal laryngeal n
- C. Glossopharyngeal n

D. Recurrent laryngeal n.

6. Which of the following nerves is located near to inferior thyroid artery?

- A. External laryngeal n
- B. Internal laryngeal n
- C. Glossopharyngeal n
- D. Recurrent laryngeal n.

7. Which of the following nerves could be injured during dissection for superior thyroid artery ?

- A. External laryngeal n
- B. Internal laryngeal n
- C. Glossopharyngeal n
- D. Recurrent laryngeal n.

8. Which of the following arteries you have to be cautious for injury of external laryngeal nerve?

- A. Superior thyroid artery
- B. Middle thyroid artery
- C. Inferior thyroid artery
- D. Internal thyroid artery

9. Why do the surgeon ligate the inferior thyroid artery away from the thyroid gland?

- A. To avoid injury of recurrent laryngeal n
- B. To avoid injury of middle thyroid vein
- C. To avoid injury of external laryngeal n.
- D. To avoid injury of internal laryngeal n.

10. Why do the surgeon ligate the superior thyroid artery within the thyroid gland?

- A To avoid injury of recurrent laryngeal n
- B. To avoid injury of middle thyroid vein
- C. To avoid injury of external laryngeal n.
- D. To avoid injury of internal laryngeal n.

11. Bilateral ligation of the inferior thyroid artery is better to be away from the thyroid gland lower pole, to avoid risk of injury to the recurrent laryngeal nerve, but there is another important risk. What is this risk?

A. Injury to thyroid tissue

B. Injury to the trachea

C. Cut the blood supply to parathyroid follicles

D. Injury to the larynx

12. Ligation of the inferior thyroid artery bilaterally is better to be away from the thyroid gland lower pole, to avoid risk of injury to the recurrent laryngeal nerve, but there is another important risk. What is this risk?

A. Disturbances of the thyroxine metabolism

B. Disturbances of calcium metabolism

C. Disturbances of Iodine metabolism

D. Disturbances of sodium metabolism

13. During thyroidectomy, although ligation of which artery is mandatory but it could require the patient to take calcium replacement therapy?

A. Superior thyroid artery

B. Middle thyroid artery

C. Inferior thyroid artery

D. Thyroids ima artery

14. Despite the risk to injure the recurrent laryngeal nerve, why do some surgeons try to ligate the inferior thyroid artery within the thyroid gland ?

A. To avoid injury to thyroid tissue

B. To avoid injury to the trachea

C. To preserve the blood supply to parathyroid gland

D. To avoid injury to the larynx

15. Why do the surgeon better retract the strap muscles in the upper part of the wound during thyroidectomy ?

A. To avoid injury of recurrent laryngeal n

B. To avoid injury of ansa cervicalis

C. To avoid injury of external laryngeal n.

D. To avoid injury of internal laryngeal n.

16. Loss of high pitched voice after thyroidectomy may be due to injury of which nerve?

A. Recurrent laryngeal n

B. Ansa cervicalis

C. External laryngeal n.

D. Internal laryngeal n.

Cardiothoracic Surgery

1. Which of the following is a cause of exudative pleural effusion?

- A. Heart failure
- B. Nephrotic syndrome
- C. Tuberculosis
- D. Hepatic failure

2. What is the cause of transudative pleural effusion?

- A. Liver cell failure
- B. Post-pneumonic
- C. Parasitic infestation
- D. Viral infection

3. What is the last stage of empyema?

- A. Catarrhal
- B. Fibro-purulent
- C. Organization
- D. Purulent

4. What is the most common cause of empyema?

- A. Post pneumonic
- B. Rupture lung abscess
- C. Post- resection
- D. Post-endoscopy

5. What is the commonest cause of lung abscess?

- A. COPD
- B. Inhalational
- C. Lung cancer

D. Empyema

6. Which of the following biochemical measurements of pleural fluid aspirate is significant for exudative pleural effusion?

A. PH =7.35

B. Glucose= 70 mg

C. Albumin= 4 gm

D. Glucose= 75 mg

7. Light's criteria for diagnosis of type of pleural effusion doesn't include which of the following parameters?

A. Pleural fluid protein

B. Serum fluid protein

C. Pleural fluid lactate

C. Pleural fluid LDH

8. Pleural fluid sample must be sent for analysis of what?

A. Cytology

B. Sodium

C. Potassium

D. Lactate

9. What is the most common lobe affected by lung abscess?

A. Rt lower lobe

B. Left upper lobe

C. Left lower lobe

D. Lingula left upper lobe

10. What is the most common pulmonary segment affected in lung abscess?

A. Apical segment of left upper lobe

B. Posterior segment of right upper lobe

C. Posterior segment of left upper lobe

D. Medial segment of right lower lobe

1. Which of the following is false about pleural effusion diagnosis?

A. Inspection: Limited chest movement

B. Palpation: Mediastinal shift

C. Percussion: Hyperresonance

D. Auscultation: Decreased air entry

3. Which of the following is NOT a type of non-small cell lung cancer?

A. Oat cell carcinoma

B. Adenocarcinoma

C. Squamous cell carcinoma

D. Large cell carcinoma

4.. Which of the following structures is NOT from the boundaries of Calot triangle?

A. Cystic duct

B. Common hepatic duct

C. Common bile duct

D. Undersurface of the liver

5. Which of the following Light's criteria help in diagnosis of exudative pleural effusion?

A. Pleural fluid protein to serum fluid protein is 0.2

A. Pleural fluid protein to serum fluid protein is 0.3

B. Pleural fluid protein to serum fluid protein is 0.4

C. Pleural fluid protein to serum fluid protein is 0.6

6. What is the definition of chronic empyema?

A. Empyema in the first 4 days

B. Empyema in the first 7 weeks

C. Empyema in the first 2 weeks

D. Lung entrapment despite adequate drainage

7. An air-fluid level appears in chest X-ray in which of the following conditions?

A. Pneumothorax

B. Lung abscess

C. Pleural effusion

D. Empyema thoracis

8. In open heart surgery, cardioplegia solution is rich in which electrolyte?

A. Sodium

B. Potassium

C. Calcium

D. Magnesium

9. What is the best conduit used in left anterior descending coronary artery stenosis during coronary artery bypass graft surgery?

A. Left internal mammary artery

B. Great saphenous vein

C. Radial artery

D. Gastroepiploic artery

Short Answer Questions

1. What is the suitable prosthetic heart valve must be inserted in a 40-year-old female with severe aortic stenosis?
2. Enumerate causes of empyema thoracis.
3. Mention indications for coronary artery bypass graft surgery.
4. Mention drugs used in management of shock.
5. Enumerate indications of cholecystectomy.
6. What is the suitable prosthetic heart valve must be inserted in a 70 year old female with severe mitral stenosis?
7. Enumerate causes of exudative pleural effusion.
8. Mention indications for coronary artery bypass graft surgery.
9. Mention indications for thyroidectomy.
10. Enumerate complications of cholecystectomy.
11. What are the causes of lung abscess?
12. What is the classification of perianal fistula?
13. What are the pathological types of breast cancer?
14. What is the classification of intestinal obstruction?
15. What are the complications of hernia?
16. What are the causes of hypovolemic shock?
17. How can you treat haematoma?
18. Mention the complications of hematoma
19. Mention investigations for breast cancer

Matching Questions

These are important differential diagnosis of abdominal pain. Match the term from column A, with its related sentence from column B and insert your answer in the table below.

Column-A	Column- B
1. Acute appendicitis	A. Rt hypochondrial pain, Fever, Jaundice
2.Acute cholangitis	B. Lumbar pain, frequency and dysuria.
3. Intestinal obstruction	C. Sharp pelvic pain, vaginal spotting, with signs of shock
4. Ureteric colic	D. Central abdominal pain then shifts to Rt. iliac fossa
5. Ruptured ectopic pregnancy	E. Abdominal colicky pain, absolute constipation, vomiting

1	2	3	4	5
D	A	E	B	C

Match the related complication of each nerve injury during thyroidectomy

1. Recurrent laryngeal nerve	A. Voice fatigue
2. External laryngeal nerve	B. Hoarseness of voice
3. Internal laryngeal nerve	C. Injury during strap muscles retraction
4. Ansa cervicalis	D. Chocking

1. B

2. A

3. D

4. C

This table contains complications of thyroidectomy, match each complication with its related term

1. Manifest tetany	A .Injury of internal jugular vein
2. Latent tetany	B.Recurrent laryngeal nerve paralysis
3. Air embolism	C.Chvostek sign
4. Chocking	D.Internal laryngeal nerve injury
5. Hoarseness of voice	E Carpopedal spasm

1. E

2. C

3. A

4. D

5. B

These are important surgical signs, match each sign with its definition.

1. Psoas sign	A . Sign of manifest tetany
2. Rovsing sign	B. Sign of latent tetany
3. Carpopedal spasm	C. Sign of lymphatic invasion by breast cancer
4. Trousseau's sign	D. Painful extension of right hip joint
5.Peau d'orange	E. Pain in Rt iliac fossa on palpation of Lt iliac fossa

1. D

2. E

3. A

4. B

5. C

These are some important surgical diversions. Match each type with its related definition

1. Cutaneous ureterostomy	A. A piece of the bowel is used to create an internal reservoir for urine
2. Suprapubic catheter	B. Done for left-sided colon cancer
3. Neobladder	C. Done for right-sided colon cancer
4. Colostomy	D. One or both ureters is directly attached to a stoma in the abdomen.
5. Ileostomy	E. It is inserted into the bladder through a small hole in the skin beneath the umbilicus.

1. D
2. E
3. A
4. B
5. C

These are important differential diagnosis of abdominal pain. Match each type with its related condition.

Column-A	Column- B
2. Acute appendicitis	A. Rt hypochondrial pain, Fever, Jaundice
3. Acute cholangitis	B. Lumbar pain, frequency and dysuria.
4. Intestinal obstruction	C. Sharp pelvic pain, vaginal spotting, with signs of shock
5. Ureteric colic	D. Central abdominal pain then shifts to Rt. iliac fossa
6. Ruptured ectopic pregnancy	E. Abdominal colicky pain, absolute constipation, vomiting

1	2	3	4	5
D	A	E	B	C

Match the term from column A, with its related sentence from column B and insert your answer in the table below

Column A	Column B
1. Cardiogenic shock	A. Type of shock may occur during doing CT scan with contrast
2. Septic shock	B. Type of shock due to release of sympathetic tone
3. Anaphylactic shock	C. Type of shock where pump failure
4. Neurogenic shock	D. Type of shock where cardiac output is increased

1	2	3	4
C	D	A	B

Match the term from column A, with its related sentence from column B and insert your answer in the table below

Column A	Column B
1. Incisional hernia	A. High predominance in female
2. Femoral hernia	B. More common in COPD patients
3. Direct inguinal hernia	C. The most common in early life
4. Indirect inguinal hernia	D. Occurs after surgical interventions within 5 years

1	2	3	4
D	A	B	C